Clinical Care-Plan Case Study: Stage 4 Sacral Pressure Injury with Early Sepsis in a Frail Older Adult with Dementia

ADN Med-Surg II | Individual take-home clinical care-plan case study | Open book

Your patient

You are the primary nurse for the patient in Medical-Surgical 3 West, room 322-2, on the 7:00 to 15:00 shift on Thursday, October 22, 2026 (hospital day 3). Open the patient in the student EHR (Student View, or the patient file from your instructor) and set the simulation clock to 0700. Everything you need is in the chart. She came in with a deep, infected pressure injury over her tailbone and early sepsis. The wound was cleaned out (debrided) at the bedside and a wound vac was placed this morning. She also has dementia, heart failure, diabetes and poor nutrition. Work the way you would at the start of a real shift.

How this case study works

This packet follows the nursing process: Assessment, Diagnosis, Planning, Implementation and Evaluation. You will also explain the pathophysiology and pharmacology behind your patient's care. Expect to spend about 5 hours. It is a take-home, individual, open-book assignment: use your textbooks, drug guide, notes and the chart.

Questions ask you to explain WHY, not only WHAT. Short, specific answers that use this patient's data earn more points than general textbook answers. Real charts are imperfect: you will be asked to catch conflicts and gaps, and to question orders before you act.

Part	Nursing process step	Estimated time
1	Assessment: Patient Data and Chart Review (Assessment)	60 min
2	Pathophysiology: The Disease Process (Assessment / Analysis of cues)	60 min
3	Pharmacology and Medication Safety (Assessment / Analysis (and safe implementation))	50 min
4	Nursing Diagnoses and Priorities (Diagnosis)	25 min
5	Planning and Implementation (Planning / Implementation)	50 min
6	Evaluation: What Happened and What Next? (Evaluation)	25 min
7	Clinical Judgment (Next-Generation NCLEX Style) (Evaluation / Clinical judgment)	30 min
	Estimated total	**5 h 0 min**

Scoring

- The packet is worth 100 points. A score of 78 or higher is passing.
- Each numbered item shows its point value. Points are given only for what is written in the box.

Writing so your pages can be scored from photos

- Write in dark pen or very dark pencil, print clearly, and stay inside the box. Write the answer in the box with the same number as the question.
- Write your name and date at the top of every page (the header prints a line for it).
- If you need more room, write "continued on the back" and put the item number on the back. Do not squeeze writing between boxes.
- Do not use abbreviations that are not on the approved list. Spell out drug names.
- When you finish: photograph EVERY page, one page per photo, on a flat surface in good light with all four corners and the page number in view. Check that every photo is readable before you upload.

Part 6 is sealed

- Do not read the sealed page at the start of Part 6 until Parts 1 to 5 are finished. Your plan must be based on what you knew at 0700.

Honor statement: I completed this work myself, using only my textbooks, notes, drug references and the patient chart.

Signature: _____ Date: _____

PART 1: ASSESSMENT: PATIENT DATA AND CHART REVIEW

Nursing process step: Assessment | About 60 minutes | 26 points

Collect and organize the data you would have at 0700. Where the chart does not document something, write "not documented." Use R and L for laterality.

1.1 Basic Patient Information *(3 pts)*

Where to look in the EHR: Patient Summary and Storyboard

Room Number:		**Age:**	
Admission Date:		**Allergies (and reactions):**	
Primary Diagnosis:		**Code Status / Decision-maker:**	

1.2 Medical Diagnoses and How They Affect the Wound *(3 pts)*

List FOUR other medical diagnoses from this patient's chart. For each, explain how it contributed to the pressure injury, slows healing, or raises her risk of complications now.

Where to look in the EHR: Chart Review > H&P; Problem list

Medical diagnosis	How it affects the pressure injury, her healing, or her care

1.3 Patient History *(2.5 pts)*

Where to look in the EHR: ED Provider Note; Hospitalist H&P; Nursing Admission Note; Case Management and PT notes

Medical History:

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

Surgical History:

Family History:

Social History, Living Situation and Function:

Pre-Hospital Story (how did she get here?):

Likely Discharge Needs:

1.4 Head-to-Toe Assessment at 0700 (use R and L for laterality) *(4 pts)*

Document the findings from the chart as of the start of your shift. If the chart does not document a finding, write "not documented." Include the PAINAD score if one is charted.

Where to look in the EHR: Flowsheets (latest assessment 10/21 2000); Nursing Progress Note 10/22 0645; vital signs 0400

Neurological / Cognition / Pain:

Cardiovascular: Pedal pulses R___ L___ Cap refill _____ Edema _____

Respiratory: breath sounds, effort, oxygen

Gastrointestinal / Nutrition:

Genitourinary:

Integumentary (wound, NPWT, perineum, heels, Braden):

Musculoskeletal / Mobility:

IV / Lines / Devices:

Psychosocial / Cultural Assessment:

Erikson Stage:

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

1.5 Vital Sign Trend *(2 pts)*

Copy the vital signs at each time. Circle every abnormal value. Then describe the trend in one sentence.

Where to look in the EHR: Flowsheets > Vital Signs (or the Vitals graph)

Vital sign	10/20 0206 (ED triage)	10/20 1200	10/21 1200	10/22 0400
Temperature				
Heart rate				
Blood pressure				
Respiratory rate				
SpO2 (%)				
Pain (0-10)				

Trend in one sentence:

1.6 Wound Assessment: Measurement and Tissue Type *(2.5 pts)*

Describe the sacral wound at each time. Give length x width x depth in centimeters, and say "not documented" if there is no entry. Name the NPIAP stage.

Where to look in the EHR: Notes (ED, Nursing Admission, Wound/Ostomy consult, Debridement note, Nursing Progress Notes 10/21 and 10/22); Flowsheets > Skin

Feature	Admission 10/20	After debridement 10/21	This morning 10/22 (NPWT)
Size L x W x depth			
Undermining / tunneling			
Tissue in wound bed and structures visible			
Drainage and odor			

Periwound skin			

1.7 Laboratory and Diagnostic Data *(4 pts)*

Choose the 8 results (labs, cultures or imaging) most important for this patient today. For each give the latest value with the normal range, the trend since 10/20, and why it matters for THIS patient.

Where to look in the EHR: Lab Results; Imaging; Chart Review > Notes

Result	Value / normal range / trend	Why is this important for this patient?

1.8 Chart Reconciliation *(5 pts)*

Real charts contain conflicts, duplicates and gaps. Find FIVE places where this chart is conflicting, duplicated, missing something, or needs clarification before you act. Say where you found each and who you would clarify with.

Where to look in the EHR: Orders, MAR, Chart Review > Notes, Lab Results, Sticky notes

What is wrong or unclear (and where)	Who you would clarify with and what you would ask

PART 2: PATHOPHYSIOLOGY: THE DISEASE PROCESS

Nursing process step: Assessment / Analysis of cues | About 60 minutes | 20 points

Explain WHY this is happening. Use your own words (plain language) and tie each idea to this patient.

2.1 Pathophysiology of a Stage 4 Pressure Injury *(5 pts)*

In plain language and your own words, explain: how pressure and shear injure tissue over the sacrum, why her risk factors made it worse, what makes this wound stage 4 (NPIAP), and why it became infected and made her septic.

2.2 Cause-and-Effect Chain *(3 pts)*

Fill in each box (one or two lines) to show how events in this patient lead from her risk factors to sepsis and what we do to interrupt the chain.

1. Why was she at high risk? (factors in THIS patient)	
▼	
2. What happened to the tissue over her sacrum?	
▼	
3. Why did the wound become infected?	

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

▼

4. How is her body responding (systemic signs)?	

▼

5. Why will healing be slow, and what complication is still possible?	

▼

6. How do debridement, NPWT, antibiotics, nutrition and offloading interrupt the chain?	

2.3 Braden Scale: Subscales and Mechanisms *(3 pts)*

Her Braden total was 12 at 0645 (the chart gives only the total: 10 on admission, 11 on 10/21 2000, 12 this morning). Assign a score to each subscale that you can defend from the chart (your six scores should total 11 to 13). Give the chart evidence, then explain how that subscale lets pressure or shear injure HER tissue.

Where to look in the EHR: Flowsheets > Skin, Neuro, Mobility, GU, GI; PT note; Orders (support surface)

Subscale (range)	Your score and chart evidence	How this lets pressure/shear injure her tissue
Sensory perception (1-4)		
Moisture (1-4)		
Activity (1-4)		
Mobility (1-4)		
Nutrition (1-4)		
Friction and shear (1-3)		

2.4 Malnutrition and Refeeding Syndrome *(2 pts)*

Why does she need more protein, calories, vitamin C and zinc? What is refeeding syndrome, why is she at risk, and what in her labs or orders shows you how to watch for it? Why is BMI 20.7 not a reliable sign that she is well nourished?

Where to look in the EHR: Nutrition consult and RD assessment; Orders (dietitian, refeeding precautions); Lab Results (phosphorus, magnesium, potassium)

2.5 Hemoglobin 7.0 -> 6.5 -> 6.4 in a Patient with HFrEF *(3 pts)*

Her hemoglobin has fallen to a critical value and no transfusion order or work-up is on the chart. Why does this matter more in HER than in a healthy adult? List three possible causes you can find in the chart, say what you would do and report, and explain what makes a transfusion riskier in her. What obvious fix would be wrong?

Where to look in the EHR: Lab Results > Hemoglobin; Notes; Orders; I&O

2.6 Recognizing Sepsis in an Older Adult with Dementia *(2 pts)*

Using her ED data, explain why she met criteria for early sepsis (use SIRS and qSOFA with her numbers). Why can sepsis be hard to see in an 84-year-old with dementia? What will you watch and report on your shift?

Where to look in the EHR: ED note and H&P (triage vitals, labs); Sticky notes; Orders (infection watch)

2.7 Treatment: How Each Piece Helps the Wound *(2 pts)*

Explain how EACH of these treats her wound problem: sharp debridement, negative pressure wound therapy (-125 mmHg continuous), antibiotics, nutrition, and the support surface with turning. Name one thing the wound vac cannot do for her.

Where to look in the EHR: Debridement and NPWT procedure notes; Wound/Ostomy and Surgery consults; Orders

PART 3: PHARMACOLOGY AND MEDICATION SAFETY

Nursing process step: Assessment / Analysis (and safe implementation) | About 50 minutes | 14 points

Use your drug guide and the MAR (click the medication name for the drug guide in the EHR). Part of your job is to question orders. Only active orders count; the LR bolus and LR maintenance orders are completed.

3.1 Medication List *(4.5 pts)*

From the MAR and orders, choose the SIX medications that matter most to you before the 0900 medication pass. List brand and generic names.

Where to look in the EHR: MAR (click the medication name for the drug guide)

Medication (brand and generic)	Drug class and mechanism (10 words or fewer)	Why is THIS patient taking it?	Key nursing check / hold parameter

3.2a Vancomycin and Piperacillin-Tazobactam *(1.5 pts)*

Vancomycin 750 mg is due at 0900 and pip-tazo continues. Her creatinine is 0.87 (it was 1.35 on admission), the 10/21 level was 12.0, the nasal MRSA PCR was negative, and the wound culture shows gram-negative rods and gram-positive cocci in pairs. What will you check before and after the dose and when will you hold and call? What is wrong with how the 10/21 "trough" was timed? What question will you ask about continuing vancomycin?

Where to look in the EHR: MAR; Orders (vancomycin, trough/AUC order); Lab Results (creatinine, vancomycin level, cultures)

3.2b Pain Assessment and the Codeine Reaction *(1.5 pts)*

She has dementia and cannot always report pain. How will you assess it (name the tool and what it scores, and her chart score)? Her orders are acetaminophen, tramadol 25 mg PRN for pain 4-6 and IV hydromorphone PRN for 7-10, and her pain is 3/10. Her codeine "allergy" is nausea and vomiting. What would you do before the next NPWT foam change (due in 48-72 hours) and how do you think about opioid choice?

Where to look in the EHR: Orders (pain assessment/premedication, analgesics); Allergies; Nursing Admission Note

3.2c The Obvious Action Is Wrong *(2 pts)*

A busy colleague offers these shortcuts for Mrs. Whitcomb. Choose FOUR of the six. For each, write why it is unsafe for HER (use her data) and what you would do instead.

(a) "She is septic and a bit dry: run a 1 L LR bolus and put her back on LR at 100 mL/hr."

(b) "She only eats half, so give every supplement now and push 1,550 calories today."

(c) "She gets agitated at night and pulls at the tubing: ask for lorazepam, or use mitts and a vest restraint."

(d) "The right hip area is red: massage it to bring the circulation back."

(e) "Put her on a donut cushion in the chair so the sacral wound touches nothing."

(f) "Lay her fully on her side, 90 degrees, to get the sacrum free, and raise the head of the bed to 60 degrees after meals."

3.2d Lisinopril, Carvedilol and Furosemide at 0900 *(1.5 pts)*

At 0900 lisinopril 20 mg, carvedilol 6.25 mg and furosemide 40 mg are due. Her BP is 115/71, HR 87, potassium 3.7, magnesium 1.7, creatinine 0.87, the hospitalist calls her "euvolemic to mildly dry," and she is on vancomycin and pip-tazo. What do you check, which drugs have hold parameters, what is missing, and what will you ask the provider before you give them?

Where to look in the EHR: MAR; Orders (hold parameters, potassium/creatinine monitoring); Flowsheets; Notes

3.3 Math From the Chart *(3 pts)*

Show your work in the box. Include units. Weight 51 kg.

Where to look in the EHR: MAR, Orders, Nutrition notes, I&O, Lab Results

a) Vancomycin 750 mg IV is supplied in 250 mL and ordered over 90 minutes. What pump rate in mL/hour? How many mg/kg is the 750 mg dose?

b) Acetaminophen is 650 mg every 6 hours. How many mg per 24 hours? The order's drug reference gives a maximum of 2 g/day for malnourished patients: by how much does her daily total exceed it, and what is the largest dose she could receive every 6 hours?

c) Ensure Enlive (350 kcal, 20 g protein) is ordered 3 times daily and Ensure Plus (350 kcal) twice daily. How many kcal per day do the supplements give? Her refeeding order says start at 10 to 20 kcal/kg/day: what is that range in kcal? Does the supplement total fit?

d) Her goal urine output is at least 0.5 mL/kg/hr. What is the minimum in mL per hour and in one 8-hour shift? Her last 24 hours intake was 2,409 mL and output 1,860 mL: what is the net balance?

PART 4: NURSING DIAGNOSES AND PRIORITIES

Nursing process step: Diagnosis | About 25 minutes | 8 points

Use NANDA-I diagnoses. Write each as problem, "related to" (cause you can treat), and "as evidenced by" (specific data). Do not use a medical diagnosis as the "related to."

4.1 Prioritized Nursing Diagnoses *(6 pts)*

Write FOUR nursing diagnoses in priority order (1 = highest).

#	NANDA-I nursing diagnosis	Related to (etiology)	As evidenced by (at least 2 specific chart findings)

4.2 Why This Order? *(2 pts)*

Explain why your #1 diagnosis is first (use ABCs, Maslow, or acuity). Name one lower-ranked diagnosis that could become the top priority and what finding would move it up.

PART 5: PLANNING AND IMPLEMENTATION

Nursing process step: Planning / Implementation | About 50 minutes | 14 points

Plan the care for the 0700-1500 shift. SMART goal = Specific, Measurable, Achievable, Relevant, Time-bound, written about the PATIENT (not the nurse).

5.1 Plan for Your Priority Diagnosis #1: _____ *(4 pts)*

Use the top diagnosis from 4.1. I = independent, C = collaborative, D = delegated.

SMART goal for this shift	Nursing interventions (at least 5: what, how often)	Rationale	I / C / D

5.2 Time-Sequenced Plan for the Shift *(3 pts)*

For each hour, list your priority actions IN ORDER. Include medications (and which you will question), wound/NPWT care, repositioning, nutrition, assessments, communication, and say why that time.

Where to look in the EHR: MAR, Orders, Notes, Sticky notes

Time	Priority actions in order	Why at this time
0700		
0800		

0900		
1000		
1100		
1200		
1300		
1400		

5.3 Calling the Provider (SBAR) *(2 pts)*

At 0745 you review the 0500 labs and the 0900 medications. Hemoglobin is 6.4 (6.5 yesterday, 7.0 on admission), there is no transfusion order or anemia work-up, enoxaparin was given at 0600, and she has HFrEF. HR 87, BP 115/71, no bleeding seen, 40-60 mL serosanguineous in the NPWT canister over 12 hours. You call Dr. Patel (hospitalist). Write what you would say.

Where to look in the EHR: Lab Results; MAR; Orders; Notes

S: Situation

B: Background

A: Assessment

R: Recommendation / request

5.4 Delegation *(2 pts)*

Mark the LOWEST level of staff who may be assigned each task for this patient.

	RN only	LPN/LVN	UAP/PCT
Turn her to a 30-degree tilt with a lift sheet and wedges, float her heels, and keep the wound vac tubing off her skin	☐	☐	☐
Compare her 0900 vital signs and urine output with her sepsis call parameters and decide whether to notify the provider	☐	☐	☐
Give the 0900 oral carvedilol and lisinopril after the RN has checked the BP and heart rate and clarified the hold parameters	☐	☐	☐
Perform the NPWT foam change and measure the wound	☐	☐	☐

5.5 Communication, Dignity, Reporting and Discharge Back to Long-Term Care *(3 pts)*

Her daughter, the health care proxy, arrives upset. Write (1) what you will tell her about the wound vac and the plan and how you will include her, (2) three ways you will protect your patient's dignity, (3) what your responsibility is because a stage 4 pressure injury developed in the long-term care facility and an order asks for neglect screening, and (4) what the receiving long-term care facility must be told and have ready before she returns.

Where to look in the EHR: Social history; Orders (social work/case management; delirium); Case Management and PT notes

SEALED PAGE. Read only after Parts 1 to 5 are complete.

Part A (from the EHR): the two chart notes written after 0700 are printed below exactly as in the chart. Part B (INSTRUCTOR ADDENDUM, NOT IN THE EHR) follows them. Part B is written by your instructor for this assignment.

10/22 08:30 Hospitalist Progress Note

Hospital Day 3

Interval events / subjective

Pain 2-3/10; comfortable; wants to sit up for meals.

- MRI pelvis (sacrum and coccyx) without contrast: Equivocal coccygeal marrow edema: osteomyelitis not excluded. Trend ESR/CRP and reassess bone at follow-up debridement.
- Application of negative pressure wound therapy (wound VAC) to sacral wound (10/22/26 0400): Wound clean with red granulation, no slough, no exposed bone or untreated infection.

Objective

- Vitals (24 h): Tmax 99.0 F; HR 87-94; BP 112-121/68-71; RR 16-18; SpO2 96-97%; pain 3/10
- Current: T 98.4 F, HR 87, BP 115/71, RR 16, SpO2 96% on room air, pain 3/10
- I&O (24 h): in 2409 mL / out 1860 mL
- Diet: 2 g sodium, consistent carbohydrate, high calorie, high protein diet; Activity: Up to chair for meals with pressure-redistribution cushion, maximum 1 hour, assist x2

Exam

- General: Awake and alert, in no acute distress.
- Cardiovascular: Sinus rhythm, rate in the 90s; S1 S2 regular, no murmur. Edema: Trace bilateral lower extremity edema.
- Respiratory: Clear bilaterally; unlabored, even.
- Abdomen: Soft, non-tender, non-distended. Bowel sounds: active in all four quadrants.
- Neurologic: Oriented to person, place and time; baseline mild forgetfulness.
- Skin: Sacral wound with NPWT: black foam collapsed, seal intact, continuous -125 mmHg, 40-60 mL serosanguineous in canister over 12 hours; periwound skin intact with barrier film; no odor or erythema.

Labs

- WBC 11.6 (H) (down from 14.5); CRP 164 (H); ESR 92 (H); Creatinine 0.87 (down from 1.05); Sodium 141 (stable from 143); Albumin 2.3 (L) (stable from 2.3); Prealbumin 9 (L) (stable from 9); Hemoglobin 6.4 (L) (stable from 6.5); Lactate 1.5 (down from 2.6)

Medications

- Scheduled: vancomycin 750 mg IV; piperacillin-tazobactam 3.375 g IV; acetaminophen 650 mg Oral; ascorbic acid 500 mg Oral; zinc sulfate 220 mg (50 mg elemental zinc) Oral; multivitamin with minerals 1 tablet Oral; high-protein oral nutrition supplement 1 bottle (350 kcal, 20 g protein) Oral; insulin glargine 20 units Subcutaneous; insulin lispro correction scale Low-dose correction scale Subcutaneous; lisinopril 20 mg Oral
- PRN: tramadol; hydromorphone; zinc oxide 20% barrier ointment; ondansetron

Assessment and plan

84-year-old female, hospital day 3.

Stage 4 sacral pressure injury with early sepsis

- Wound red and granulating without slough; afebrile; WBC 12.2 and CRP falling; final culture pending (E. coli, Enterococcus likely). MRI equivocal for early coccygeal osteomyelitis: ESR/CRP trend and reassess bone.
- NPWT -125 mmHg continuous; foam change every 48-72 hours with premedication.
- Stop vancomycin when the final culture shows no MRSA; piperacillin-tazobactam to finish about 4 days after debridement.
- Up to chair for meals 1 hour with cushion; PT/OT.
- Case management: SNF/home health and NPWT equipment; family or caregiver teaching.

Type 2 diabetes mellitus

- Basal glargine with ACHS correction lispro; goal glucose 140-180 mg/dL.
- Metformin held while inpatient.

- Hypoglycemia protocol in place.
Hypertension
- Continue home antihypertensive; BP goal below 140/90.
Heart failure with reduced ejection fraction
- Euvolemic to mildly dry on exam; continue beta blocker; judicious IV fluids with daily weights.
- Continue home diuretic.
Dementia
- Delirium precautions; avoid benzodiazepines/anticholinergics; family at bedside if possible.
- Capacity limited; involve surrogate decision maker.
Malnutrition / underweight
- Dietitian-guided high-calorie, high-protein diet with supplements; advance calories slowly.
- Refeeding labs (Phos, Mg, K) daily x3 days; thiamine before feeding.
- Pressure injury prevention; weekly weights.
Urinary incontinence (overactive bladder)
- Scheduled toileting and call-light assistance; avoid catheter.
- Skin checks for incontinence-associated dermatitis; fall precautions for urgency.
Frailty
- Early mobility, nutrition support, delirium and fall precautions.
- Start low, go slow with medications; goals-of-care discussion.
Disposition
- Discharge in 2-3 days.
- Code status: Full Code.

10/22 09:00 Physical Therapy Treatment Note

Session
- Therapeutic exercise, gait and transfer training; total assist with mechanical lift for transfers.
- Gait bed to chair with a lift (no ambulation); vitals stable with activity (T 98.4 F, HR 87, BP 115/71, RR 16, SpO2 96% on room air, pain 3/10).
Progress / plan
- Progressing toward discharge goals; continue daily PT.

PART B INSTRUCTOR ADDENDUM (not in the EHR) 0915 Daughter at the bedside

- She is back in bed on the low-air-loss mattress after PT. Her daughter (health care proxy) arrives, tearful. She says: "Mom usually knows me and her own name, not the year. She is worse around 5 in the evening." She says she first saw a red spot on her mother's tailbone about three weeks ago, told the facility nurse and was told "we are turning her." She asks if the wound vac will hurt, whether the facility can care for it, and says "please do not let them sedate her."

0930 Provider call

- Dr. Patel is reached about the hemoglobin of 6.4. Orders: type and screen; transfuse 1 unit packed red cells over 3 hours; add iron studies, B12, folate and reticulocyte count to ONE blood draw; repeat hemoglobin 1 hour after the unit; review tomorrow's enoxaparin dose after the repeat result. The daughter agrees to the transfusion and signs consent.

1015 Transfusion started

- One unit of red cells is started through the left antecubital IV at 100 mL/hour with 0.9% saline. Before starting: lungs clear, trace leg edema, NPWT canister 20 mL serosanguineous since 0700, seal intact.

1130 75 minutes into the unit (about 125 mL infused)

- She says "I can't breathe," is coughing, restless and pulling at the wound vac tubing and trying to climb over the bed rail. New crackles at both lung bases. The daughter is at the bedside.

1135-1215 Assume you stopped the transfusion and called

- The unit was stopped at 1135 after about 125 mL. The provider orders furosemide 40 mg IV (given 1150) and oxygen 2 L by nasal cannula, and the blood bank is notified. At 1215 she is calmer with her daughter present; the tubing is re-secured and the NPWT seal is intact.

1240-1400 Rest of the shift

- 1240 incontinent of loose stool, cleaned, barrier ointment applied, seal re-checked and intact. Glucose 168 before lunch. Ate 25% of lunch. Turned to a 30-degree right tilt at 1300. NPWT canister total since 0700: 60 mL serosanguineous. The repeat hemoglobin is pending at 1400 and pip-tazo is due at 1400.

Time	HR	BP	RR	SpO2	Temp	Other
0400 (chart)	87	115/71	16	96% room air	98.4 F	pain 3/10
0900 (chart, PT note)	87	115/71	16	96% room air	98.4 F	pain 3/10
1015 instructor	88	118/70	16	96% room air	98.5 F	lungs clear
1130 instructor	108	148/88	26	89% room air	98.7 F	crackles, restless, PAINAD 6
1215 instructor	98	136/78	20	94% on 2 L	98.8 F	calmer
1400 instructor	92	132/76	18	95% on 2 L	98.6 F	repeat Hgb pending

Your Part 1-5 answers must not change. Use these data in 6.1 to 6.3.

PART 6: EVALUATION: WHAT HAPPENED AND WHAT NEXT?

Nursing process step: Evaluation | About 25 minutes | 8 points

Open the sealed page. It shows the real chart notes written after 0700 and then events written by your instructor (the instructor events are NOT in the EHR; use them as if you had charted them). Do not change your Part 1-5 answers.

6.1 Evaluating Your Goals *(3 pts)*

Copy the SMART goal from 5.1 and one more goal you set in 5.2 (for example for skin, NPWT, nutrition or fluid balance). For each, use data from the sealed page to say whether it was met, partly met, or not met, and what you will change.

Goal (copy from Part 5)	Data from the sealed page that you used	Met / Partly / Not	Revised plan

6.2 New Problem and Priority Actions *(3 pts)*

Name the MOST urgent problem at 1130, give the data that show it, and write your priority nursing actions in order. Include what you do about her breathing AND her sacral wound position, her agitation, and who you notify.

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

6.3 Focused Nursing Note *(2 pts)*

Write a focused nursing note for the 1130 event (DAR, SOAP, or your school's format): data, action, and response, with the plan.

PART 7: CLINICAL JUDGMENT (NEXT-GENERATION NCLEX STYLE)

Nursing process step: Evaluation / Clinical judgment | About 30 minutes | 10 points

Answer by marking the boxes. These items use the same patient. Some scenarios are hypothetical and say so.

7.1 Select All That Apply *(2 pts)*

Which nursing actions are correct for this patient today?

Select ALL that apply.

- ☐ Turn her on a 30-degree tilt with wedges, alternating left, back-tilt and right, every 2 hours.
- ☐ Massage the reddened area over her right hip to improve circulation.
- ☐ Re-check the wound vac seal after each turn and after each incontinence episode.
- ☐ Place her on a donut-shaped cushion in the chair.
- ☐ Lay her directly on her side at 90 degrees so the sacrum bears no weight.
- ☐ Keep the head of the bed at 30 degrees or less except at meals.
- ☐ Apply zinc oxide barrier ointment to the buttocks and perineum, not into the wound.
- ☐ Add two extra layers of linen and a plastic-backed pad on the air mattress for easy cleanup.

7.2 Trend: Better, Same or Worse? *(2 pts)*

Judge each trend from 10/20 to 10/22 clinically: is she moving toward safety or away? A value can still be in the normal range and be getting worse. Mark one box per row.

	Improving	Stable	Getting worse
WBC 16.7 -> 14.5 -> 11.6 K/uL	☐	☐	☐
Hemoglobin 7.0 -> 6.5 -> 6.4 g/dL	☐	☐	☐
Creatinine 1.35 -> 1.05 -> 0.87 mg/dL	☐	☐	☐
Glucose 184 -> 176 -> 182 mg/dL	☐	☐	☐
Phosphorus 3.4 -> 3.2 -> 2.9 mg/dL (normal 2.5-4.5) while intake is rising	☐	☐	☐

7.3 Bowtie *(2 pts)*

HYPOTHETICAL day 5: her calories were advanced quickly to the full goal. She is now weak and confused. HR 124 and irregular, RR 26, SpO2 91% on room air, new pitting edema of both legs, and fine crackles at the bases. Phosphorus 1.6 mg/dL, potassium 3.0 mmol/L, magnesium 1.2 mg/dL.

Actions to Take (select 2)	Condition the patient is most likely experiencing (select 1)	Parameters to Monitor (select 2)

☐ Hold further calorie advancement, notify the provider now and ask for electrolyte replacement ☐ Place her on continuous cardiac monitoring and ask for a 12-lead ECG ☐ Give an extra supplement bottle to restore her strength ☐ Give a 1 L fluid bolus for weakness ☐ Give the scheduled furosemide for the edema without checking the electrolytes	☐ Refeeding syndrome ☐ Hypovolemic shock ☐ Hypoglycemia ☐ Opioid toxicity	☐ Serum phosphorus, potassium and magnesium ☐ Heart rate and rhythm ☐ Wound drainage color ☐ Braden score ☐ Urine color

7.4 Recognize Cues *(2 pts)*

Excerpt from a HYPOTHETICAL day-shift report on a later day. Which statements need follow-up NOW?

Draw a line under each statement that needs follow-up now. Then write the item numbers in the box below.

(1) Pain 3/10 at rest; PAINAD 2 during the turn.

(2) Temperature 101.8 F, HR 116, and new confusion since 0200; yesterday she was oriented to person and place.

(3) Ate 60% of breakfast and drank two supplements.

(4) NPWT canister has 90 mL of serosanguineous fluid over 12 hours; seal intact.

(5) A red area over the left hip does not blanch when pressed.

(6) Bright red fluid is filling the NPWT tubing; 150 mL in the canister in the last hour.

(7) Phosphorus 2.3 mg/dL (2.9 two days ago); calorie advance is due today.

(8) Glucose 168 before lunch.

Item numbers that need follow-up:

7.5 Priority Actions *(2 pts)*

HYPOTHETICAL: while on enoxaparin she suddenly has bright red blood filling the wound vac tubing and canister. HR 124, BP 86/50, pale and sweaty. Number the actions in the order you would do them.

Number the actions 1 (first) to 5 (last).

[___] Turn off the wound vac, clamp the tubing, and leave the dressing in place.

[___] Apply firm direct pressure over the dressing with a gloved hand and stay with her.

[___] Call the rapid response team or surgeon with SBAR.

[___] Apply oxygen, attach monitors, check IV access, and prepare for labs, type and cross, and possible blood.

[___] Hold the next enoxaparin dose, measure the blood loss in the canister and document.